

ENCOUNTER 3 OF 5

Date of Service: 2024-12-21 | Type: SURGICAL | Specialty: Orthopedic Surgery | Setting: Inpatient | Acuity: LOW

Attending Provider:	Dr. Anita Reyes, MD	Location:	New Amandaville Regional Medical Center - Main OR
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Chief Complaint

Numbness and tingling in the left toes, now with MRI-confirmed compressive lesion, presenting for surgical excision.

History of Present Illness

The patient is a 45-year-old female who presents with a chief complaint of numbness and tingling in the left toes. The onset of symptoms was gradual, beginning approximately 6 months prior to her initial evaluation earlier this year. The location of the numbness and tingling is primarily in the left toes, but occasionally radiates to the left foot. The duration of symptoms is constant, with the patient experiencing numbness and tingling throughout the day. The character of the numbness and tingling is described as a pins-and-needles sensation. Aggravating factors include prolonged standing or walking, while relieving factors include rest and elevation of the left leg. The severity of symptoms is mild to moderate, with the patient rating the pain as 3/10.

Prior to presentation for surgery, the patient tried over-the-counter pain medications, including ibuprofen and acetaminophen, with minimal relief. The patient also underwent a trial of physical therapy, which provided some relief but did not completely resolve symptoms. Subsequent nerve conduction studies and electromyography demonstrated focal slowing of the posterior tibial nerve without denervation, and MRI of the left foot/ankle obtained thereafter confirmed a small ganglion cyst compressing the posterior tibial nerve at the level of the tarsal tunnel. Given the confirmed structural, compressive etiology and persistent symptoms despite conservative management, the patient elected to proceed with surgical excision. Associated symptoms present include mild swelling in the left foot and occasional shooting pains in the left leg. Associated symptoms absent include fever, chills, and redness.

Review of Systems (12-point)

System	Findings
Constitutional	Denies fever, chills, or unintentional weight loss.
HEENT	History of seasonal allergies; denies vision changes or hearing loss.
Cardiovascular	History of hypertension, well controlled; denies chest pain or palpitations.
Respiratory	History of mild intermittent asthma; denies current dyspnea or wheeze.
Gastrointestinal	Reports occasional constipation; denies nausea, vomiting, or GI bleeding.
Genitourinary	Reports occasional urinary frequency; denies dysuria or hematuria.
Musculoskeletal	Reports mild left knee stiffness; denies acute joint swelling elsewhere.
Neurological	Reports numbness/tingling in left toes; denies headache, seizure, or syncope.
Psychiatric	History of generalized anxiety, stable on no pharmacotherapy; denies SI/HI.
Endocrine	History of hypothyroidism, on levothyroxine; denies heat/cold intolerance.
Integumentary	Reports occasional dry skin; denies rash or new lesions.
Hematologic/Lymphatic	History of mild anemia; denies easy bruising or lymphadenopathy.

Past Medical History

Chronic conditions include essential hypertension (dx. 2016), mild intermittent asthma (dx. childhood), and primary hypothyroidism (dx. 2019, on levothyroxine replacement). Surgical history is notable for a low-transverse cesarean section (2010) and a tonsillectomy (childhood). The patient was hospitalized previously for community-acquired pneumonia (2015) and following a motor vehicle collision (2018) with no long-term sequelae reported at that time.

Family History

Mother: hypertension, type 2 diabetes mellitus, living. Father: coronary artery disease, myocardial infarction at age 61, living. One sibling (sister): hypothyroidism. No known family history of malignancy, bleeding disorders, or hereditary neuropathy.

Social History

The patient is a non-smoker with no history of tobacco use and denies use of illicit substances. She reports occasional social alcohol use (1-2 drinks per week) and regular caffeine intake (1-2 cups of coffee daily). She works as an administrative coordinator, a role that requires prolonged standing and walking throughout the day. She is married and lives with her spouse and two children. She reports a generally sedentary exercise pattern with occasional weekend walking.

Current Medications

Medication	Dose	Route	Frequency	Indication
Aspirin	81 mg	PO	Daily	Cardioprotective
Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain
Pre-op Cefazolin	2 g	IV	Single pre-op dose	Surgical prophylaxis

Allergies

No known drug allergies (NKDA). No known food or environmental allergies of clinical significance documented.

Vital Signs

Parameter	Value	Parameter	Value
Blood Pressure	125/66 mmHg	Heart Rate	63 bpm
Temperature	98.0 F	Respiratory Rate	16 breaths/min
Pain Score	3/10	SpO2	96% RA
Weight	153 lbs	Height	5 ft 5 in
BMI	25.5 kg/m2		

Pre-Operative Physical Examination

General

Well-developed, well-nourished female in no acute distress.

Cardiovascular

Regular rate and rhythm, S1/S2 normal, no murmurs/gallops/rubs, no JVD, 2+ pedal pulses.

Respiratory

Clear to auscultation bilaterally, no wheezes/rhonchi/crackles, symmetric expansion.

Neurological

Intact sensation to light touch and pinprick in the right lower extremity, decreased sensation to light touch and pinprick in the left lower extremity, most pronounced over the dorsal left toes.

Musculoskeletal

Mild stiffness in the left knee, full range of motion in the left hip and ankle.

Skin

No rash or lesions noted.

Radiology Report - MRI Left Foot/Ankle (Final)

Exam Date:	2024-11-14
Ordering Provider:	Marcus Webb, MD
Interpreting Radiologist:	Dr. Felicia Grant, MD
Clinical History:	Chronic left toe numbness/tingling, abnormal NCS of posterior tibial nerve

Technique

Multiplanar, multisequence MRI of the left foot and ankle was performed without and with intravenous gadolinium contrast.

Findings

There is a well-circumscribed, T2-hyperintense, non-enhancing cystic lesion measuring approximately 1.1 x 0.8 x 0.7 cm situated within the tarsal tunnel, intimately associated with the tendon sheath of the flexor digitorum longus and abutting the posterior tibial nerve, which appears mildly displaced and flattened at this level. No abnormal enhancement is seen within the lesion to suggest a solid or malignant component, findings most consistent with a ganglion cyst. The remainder of the posterior tibial nerve course is unremarkable. No plantar fasciitis, Achilles tendinopathy, or significant tenosynovitis is identified. The osseous structures of the foot and ankle are unremarkable, without stress reaction, fracture, or marrow edema. The visualized musculature is unremarkable. No joint effusion of the ankle or subtalar joint.

Impression

1. Approximately 1.1 cm cystic lesion within the tarsal tunnel, consistent with a ganglion cyst, resulting in mild mass effect upon and displacement of the posterior tibial nerve, correlating with the patient's electrodiagnostic findings and clinical symptoms. 2. No other acute osseous or soft tissue abnormality of the left foot/ankle.

Pre-Operative Diagnostic Summary

Test	Result	Reference Range	Flag
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CBC	Within normal limits	See panel	Normal
CMP	Within normal limits	See panel	Normal
Lipid Panel	Within normal limits	See panel	Normal
MRI, Left Foot/Ankle	Ganglion cyst compressing posterior tibial nerve, tarsal tunnel	N/A	Abnormal
Type & Screen	O positive, antibody screen negative	N/A	Normal

Assessment

The patient has a diagnosis of chronic left foot numbness and tingling secondary to compression of the posterior tibial nerve by a ganglion cyst at the tarsal tunnel. The patient also has a history of hypertension, asthma, and hypothyroidism, all of which are currently controlled and pose no contraindication to surgery.

Pre-Operative Plan

1. Proceed with left foot exploration and excision of ganglion cyst under general anesthesia.
2. Administer pre-operative IV cefazolin for surgical site infection prophylaxis.
3. Continue home medications perioperatively per anesthesia protocol; hold as directed morning of surgery.
4. Post-operative pain management, wound care, and follow-up in 7 days.

Operative Report

Pre-operative Diagnosis:	Chronic left foot numbness and tingling secondary to tarsal tunnel ganglion cyst
Post-operative Diagnosis:	Same as pre-operative diagnosis
Procedure Performed:	Left foot exploration with excision of ganglion cyst, posterior tibial nerve decompression
Surgeon:	Anita Reyes, MD
Assistant:	Daniel Ho, PA-C
Anesthesia:	General anesthesia, endotracheal tube, per Dr. Priya Nair, MD (Anesthesiology)
Anesthesia Start / End:	07:42 / 09:05
Positioning:	Supine, with a pillow under the left leg; tourniquet applied to left thigh
EBL:	50 mL
IV Fluids:	800 mL Lactated Ringer's
Specimens:	Ganglion cyst, sent to pathology for evaluation
Complications:	None

Technique

After confirmation of correct patient and surgical site per institutional time-out protocol, the patient was brought to the operating room and placed in the supine position with a pillow under the left leg. General anesthesia was induced without complication. A pneumatic tourniquet was applied to the left thigh. The left lower extremity was prepped and draped in standard sterile fashion. A curvilinear incision was made over the medial aspect of the ankle, centered over the tarsal tunnel. Dissection was carried down through subcutaneous tissue with careful hemostasis using electrocautery. The flexor retinaculum was identified and incised longitudinally to expose the contents of the tarsal tunnel. Dissection continued down to the level of the posterior tibial nerve, which was found to be compressed by a small ganglion cyst arising adjacent to the flexor digitorum longus tendon sheath. A nerve stimulator was used intermittently to confirm nerve integrity throughout the dissection. The ganglion cyst was carefully dissected free from the surrounding neurovascular structures using a #15 scalpel and Metzenbaum scissors and was excised in its entirety without violation of the nerve epineurium. The posterior tibial nerve was inspected and found to be intact and well-perfused following decompression. Hemostasis was confirmed after tourniquet deflation. The flexor retinaculum was re-approximated loosely with 4-0 Vicryl suture to avoid re-tethering the nerve. The subcutaneous tissue was closed in layers with 4-0 Vicryl suture, and the skin was closed with 4-0 Vicryl in a subcuticular fashion. A sterile dressing was applied, followed by a soft bulky compressive wrap.

Instruments/Implants Used

#15 scalpel, Metzenbaum scissors, nerve stimulator, bipolar electrocautery, 4-0 Vicryl suture.

Disposition

The patient tolerated the procedure well and was transferred to the post-anesthesia care unit (PACU) in stable condition, awake, extubated, and hemodynamically stable.

Post-Anesthesia Care Unit (PACU) Note

The patient arrived to PACU at 09:12 awake, alert, and hemodynamically stable. Initial PACU vitals: BP 118/68 mmHg, HR 72 bpm, RR 14/min, SpO2 99% on 2L nasal cannula, pain 2/10 at the surgical site. Left foot dressing clean, dry, and intact with no evidence of active bleeding. Capillary refill less than 2 seconds at the toes, toes warm and well-perfused, sensation grossly intact to light touch distal to the surgical site (improved compared to pre-operative baseline for the mid-foot, with expected surgical-site numbness at the incision itself). The patient was monitored for 90 minutes per protocol with stable vitals throughout and was discharged from PACU to the inpatient orthopedic floor for overnight

observation given planned inpatient admission status.

Discharge / DME Plan

The patient will be discharged home with a follow-up appointment in 7 days. The patient will be given a prescription for pain management and will be instructed to elevate the left leg and avoid heavy lifting or bending. She will be provided a post-operative surgical shoe and instructed on dressing care and signs of infection to monitor for at home.

Follow-up needed: Yes, in 7 days.

Surgical / Inpatient Care Team

Name	Role	Contact
Anita Reyes, MD	Attending Orthopedic Surgeon	555-201-4410
Daniel Ho, PA-C	First Assist	555-201-4415
Priya Nair, MD	Attending Anesthesiologist	555-201-8810
Wendy Ostrander, CRNA	Anesthesia (nursing)	555-201-8815
Marissa Chu, RN	Circulating Nurse	555-201-8820
Todd Farrow, CST	Scrub Technician	555-201-8825
Lauren Best, RN	PACU Nurse	555-201-8830
Nina Osei, RN	Inpatient Floor Nurse (overnight)	555-201-8840

Inpatient Nursing Flowsheet - Overnight Observation

Time	BP	HR	RR	Temp	SpO2	Pain
09:12	118/68	72	14	97.5F	99% 2L	2/10
11:00	116/70	70	14	97.8F	98% RA	2/10
15:00	120/72	68	15	98.0F	97% RA	3/10
19:00	119/70	66	15	98.1F	97% RA	2/10
23:00	121/74	68	16	98.0F	97% RA	2/10
03:00	117/69	64	15	97.7F	98% RA	1/10
06:30	122/71	66	16	97.9F	98% RA	2/10

The patient remained hemodynamically stable throughout the overnight observation period with well-controlled pain on the standing acetaminophen and PRN oxycodone-acetaminophen regimen. Left foot dressing remained clean, dry, and intact on all checks. Neurovascular checks of the left foot were performed hourly for the first four hours and then every four hours thereafter, with toes consistently warm, well-perfused, and with brisk capillary refill throughout. The patient ambulated to the bathroom with assistance twice overnight without difficulty and tolerated a regular diet. She was cleared for discharge by the surgical team on morning rounds.

Anesthesia Record Summary

ASA Classification:	ASA II
Airway:	Endotracheal tube, Grade I view, atraumatic intubation
Anesthetic Agents:	Propofol induction, sevoflurane maintenance, fentanyl, ondansetron
Tourniquet Time:	42 minutes, left thigh
Fluids:	800 mL Lactated Ringer's
Estimated Blood Loss:	50 mL
Post-op Nausea/Vomiting:	None

Electronically signed by: **Anita Reyes, MD**

Department: Orthopedic Surgery

Signed: 2024-12-21 11:20 AM